

Conrad Ruckert

NAME

39400-480

REG. NO.

FEDERAL CORRECTIONAL INSTITUTION
P.O. BOX 1000
MILAN, MICHIGAN 48160

RECEIVED
FEB 18 2026

CLERK'S OFFICE
U.S. DISTRICT COURT

METROPLEX MI 480

13 FEB 2026 PM 6 L



United States District Court
Eastern District of Michigan
Attn: Court Clerk
231 West Lafayette Boulevard, Ste F1
Detroit, MI 48226

48226-270099



EXHIBIT D: MEDICAL EVIDENCE OF PRIOR VIOLENCE AND POLICE INTERVENTION

To the Honorable Judge:

Because I am filing this petition as an immediate *ex parte* emergency to secure my physical safety prior to the Respondent's March 2nd release, I have not attached the formal Plymouth Police Department incident report from May 4, 2023.

Instead, I am attaching the Respondent's VA Emergency Department medical records from that exact week for transparency.

Because this is a lengthy medical file, **I direct the Court's attention specifically to the clinical notes dated May 4, 2023**, which serve as absolute proof that law enforcement responded to our residence for domestic violence and validate my current fear for my life:

1. Proof of the Police Ultimatum for Aggression In the Mental Health Consult note dated May 4, 2023 (authored by Nancy Bigelow, NP), the medical record explicitly documents the police intervention, stating:

"Police were called to the residence and Pt reports they gave him a choice of going to jail or coming to the ED."

2. Proof of the Domestic Altercation In the Emergency Department Flowsheet dated May 4, 2023 (Authored by Brandon Templeton, RN), the triage note explicitly states:

"Patient states he was in an altercation with wife last night that spilled over into today when PD were called."

The Respondent chose the hospital to avoid arrest. This establishes a documented, medical baseline of physical violence that proves my physical safety is in imminent danger upon his release.

LOCAL TITLE: EMERGENCY DEPARTMENT MENTAL HEALTH CONSULT

STANDARD TITLE: EMERGENCY DEPT NOTE

DATE OF NOTE: MAY 04, 2023@15:41 ENTRY DATE: MAY 04, 2023@15:41:31

AUTHOR: BIGELOW,NANCY MARIE EXP COSIGNER:

URGENCY: STATUS: COMPLETED

*** EMERGENCY DEPARTMENT MENTAL HEALTH CONSULT Has ADDENDA ***

EMERGENCY DEPT MENTAL HEALTH NOTE

PATIENT IDENTIFYING DATA

Name: ROCKENHAUS,CONRAD ALAN

Age: 41

Gender: MALE

Ethnicity: WHITE

Marital status: DIVORCED

Total time: 90 min including F2F with Pt and care coordination.

Pt was interviewed in the ED together with Nancy Bigelow, NP.

DETAILS OF PRESENTING PROBLEM:

Pt is a 41 y/o male with PPHx including Opioid use d/o on MAT, Bipolar d/o, PTSD, anxiety and depression with multiple past suicide attempts and psychiatric hospitalizations (most recently on AIMH April 26, 2023 to May 1, 2023), who presented to the ED via EMS after an alleged argument with his fiancée. Police were called to the residence and Pt reports they gave him a choice of going to jail or coming to the ED.

On interview today he denies any mood disturbance or thoughts of self-harm, suicide, or thoughts of harming others. Pt endorses stable mood since discharge from AIMH on 5-1-2023. He reports more stressors in his relationship since starting couples therapy on 5-2-2023, stating "I feel like she doesn't validate my stressors . . .and she doesn't like when I point this out to the therapist".

The alleged altercation happened today when he says he had a seizure and his fiancée got upset and started "yelling at me" and that "I can't be your caretaker" because she could lose her job. The fiancée then insisted he lie down after his "seizure", and when he awakened, she called him "pathetic" and "a limp-dick man" and wanted him to leave. The Pt reports he then said "I'm not leaving because I pay the rent".

At that point the fiancée called 911, stating that she told the police that he pushed her which Pt denies. Pt then stated that he also called 911 "so they could hear my side of the story". When the police arrived Pt states the police believed that he did not push the fiancée but gave him the ultimatum of "going to jail or going to the hospital to appease her". Reports the police told him he could return back to the residence after being evaluated.

MH team informed the Pt that we will need to verify with the police department that he may return home or what the appropriate steps will be from ED. Pt is agreeable and states he lives in Plymouth, MI and the Plymouth Police Department was called. The Police department verified they have no concerns for him to be discharged and return to his residence.

Denies any issues with sleep or appetite. Reports compliance with medication regimen and intention to attend post discharge appointments; Pt also reports he is planning on attending all f/u appointments including appointment tomorrow.

PSYCHIATRIC HISTORY:

- Per chart, Dx include Bipolar I (mood swings & over-spending), opioid use d/o related to chronic back pain (now on suboxone), PTSD, depression, anxiety, possible delusional d/o with paranoid and sometimes grandiose delusions.
- Previous treatment in TX, recently established care in SUD-C here.
- Multiple past psych admissions (at least 12), most recently on AIMH from 4-26-2023 to 5-1-2023.
- Multiple past psych med trials including Ambien, Klonopin, Valium, Ativan, Eszopiclone, Zoloft, Remeron, Effexor, Cymbalta, Doxepin, Nortriptyline, Seroquel, Haldol, Risperdal consta, Geodon, Depakote, Lithium, Buspar, Hydroxyzine, Propranolol, Prazosin, Trazodone, Suboxone.
- Per chart, multiple suicide attempts. Pt discussed today's attempt via lying on railroad tracks, fiance found him by tracking his location on his phone then brought him to the ED. Pt reported the most recent attempt prior to today was in Oct 2022 - ingested ~50 Metformin tablets in the context of difficulty adjusting to civilian life after incarceration, argument w/ fianc?e and opiate use.

SUBSTANCE USE HISTORY:

- OUD on MAT with buprenorphine
- denies cannabis use "I quit" since discharging from AIMH unit on 5-1-2023
- denies etoh, denies illicit drug use
- occasional adderall from a friend who has a prescription

PSYCHOSOCIAL HISTORY AND STRESSORS:

Divorced, 3 children (notes from 4-27-2023 indicated Pt has 2 children). Recently moved from TX to MI and lives with his fiance, they've been together x3 years. Current income from 100% VA SC. Per chart, Pt has a BA in computer science, past employment- eb/security/computer consulting business. Currently on supervised release for 3 years after ~40 months of incarceration for computer hacking. Pt cites his fiance and his mother as main social supports. Independent w/ ADLs. Denied access to firearms.

Military Service - Navy 2003-2015

Percent SC: 100

Rated disabilities:

SCARS 0% SC

BIPOLAR DISORDER 70% SC

TRAUMATIC BRAIN DISEASE 10% SC

LIMITED FLEXION OF THIGH 0% SC

LIMITED MOTION OF ARM 0% SC

HYPERTENSIVE VASCULAR DISEASE 0% SC

SCARS 0% SC

SCARS 0% SC

LIMITED MOTION OF ARM 0% SC

SLEEP APNEA SYNDROMES 50% SC

MIGRAINE HEADACHES 30% SC

LIMITED FLEXION OF THIGH 10% SC

SCARS 0% SC
LUMBOSACRAL OR CERVICAL STRAIN 0% SC
PARALYSIS OF SCIATIC NERVE 10% SC
LUMBOSACRAL OR CERVICAL STRAIN 10% SC
IRRITABLE COLON 0% SC
LIMITED FLEXION OF KNEE 10% SC
TINNITUS 10% SC
SEIZURE DISORDER 100% SC

MEDICAL HISTORY:

Per chart:

- Seizure disorder
- Sleep apnea, not using CPAP ("ex-wife stole it")
- Migraine
- Sciatica
- Chronic back pain
- Tendinitis
- Hypertension
- Neck pain
- Atopic dermatitis
- Hearing loss
- Morbid obesity
- Diabetes
- Allergic rhinitis
- Pulmonary vein injury
- GERD

Active Outpatient Medications (including Supplies):

-
- 1) ACETAMINOPHEN 250/ASA 250/CAFF 65MG TAB TAKE 2 ACTIVE
TABLETS BY MOUTH EVERY DAY AS NEEDED FOR HEADACHE
 - 2) ACETAMINOPHEN 500MG TAB TAKE 1 TABLET BY MOUTH TWICE ACTIVE
A DAY AS NEEDED FOR PAIN
 - 3) BUPRENORPHINE 8MG/NALOXONE 2MG SL TAB DISSOLVE 1 ACTIVE
TABLET UNDER THE TONGUE EVERY MORNING - THIS RX FOR
5-1-23 TO 5-16-23
 - 4) FAMOTIDINE 20MG TAB TAKE ONE TABLET BY MOUTH TWICE A ACTIVE
DAY FOR HEARTBURN
 - 5) FOLIC ACID 1MG TAB TAKE ONE TABLET BY MOUTH ONCE ACTIVE
DAILY
 - 6) LITHIUM CARBONATE 300MG CAP TAKE FOUR CAPSULES BY ACTIVE
MOUTH AT BEDTIME FOR MENTAL HEALTH
 - 7) PILL SPLITTER-EA USE SPLITTER -- AS DIRECTED TO SPLIT ACTIVE
TABLETS
 - 8) PRAZOSIN HCL 1MG CAP TAKE THREE CAPSULES BY MOUTH ACTIVE
TWICE A DAY FOR NIGHTMARES OR SLEEP DISRUPTION
 - 9) RISPERIDONE 1MG TAB TAKE ONE-HALF TABLET BY MOUTH ACTIVE
THREE TIMES A DAY AS NEEDED FOR ANXIETY AND
DIFFICULTY SLEEPING
 - 10) TRAZODONE HCL 100MG TAB TAKE ONE TABLET BY MOUTH AT ACTIVE
BEDTIME FOR SLEEP
 - 11) VITAMIN B COMPLEX/VITAMIN C CAP/TAB TAKE 1 TABLET BY ACTIVE
MOUTH ONCE DAILY FOR VITAMIN SUPPLEMENT

MENTAL STATUS EXAMINATION:

Alert: sitting up in bed in VA attire good hygiene, good eye contact, pleasant and cooperative in interview

Orientation: A & O x 4

Psychomotor: no psychomotor agitation/retardation. No tremors or tics noted.

Speech: normal rate, rhythm, loud volume x times

Mood: "fine"

Affect: broad, reactive; mood congruent

Thought processes: overall logical, linear, goal-directed

Concentration: appropriate to interview; no deficits noted

Thought content: Denies SI/HI intent or plan. Denies SIB thoughts/urges.

Hallucinations: Denies AVH. No evidence of responding to internal stimuli or thought blocking.

Delusions: No paranoid or delusional statements elicited or expressed.

Insight: limited-to-fair

Judgement: limited-to-fair

SUICIDE RISK:

See CSRE

HOMICIDE/VIOLENCE RISK: No. Plymouth Police Department contacted by MH team to verify they have no safety concerns to discharge Pt to return to the residence; Plymouth PD have no concerns for Pt to be discharged.

DSM V DIAGNOSES:

-mood disorder unspecified d/o

-bipolar d/o by history

-OUD severe on MAT with buprenorphine

-chronic PTSD by history

-Delusional d/o by history

-cannabis use daily (recently quit)

IMPRESSION:

Pt is a 41 y/o male with psychiatric history of bipolar d/o, chronic PTSD, delusional d/o, OUD severe on MAT, past suicide attempts (most recent report of S.A is lying on railroad tracks on 4-26-2023), numerous inpatient psychiatric admissions (most recent AIMH 4-26-2023 to 5-1-2023), has received MH treatment at the inpatient and outpatient levels of care who presents to AAVAED via EMS for purported SI after argument with fiancée.

Pt denies low mood, SI, or HI now or at any time since discharge from AIMH unit on 5-1-2023 and reports that the police gave him the choice of going to jail or going to get evaluated in the ED after 911 was called after argument with fiancée. Pt chose to come to ED to be evaluated and "appease" his fiancée. The Plymouth Police Dept. was contacted by the ED MH team and report each party said they pushed the other and they also confirm there are no safety concerns for the Pt to return to residence.

Pt provided his version of events that led to ED presentation and in summary: he and fiancée had argument today after he had a "seizure" and she said she could not be his caretaker. The argument escalated to the fiancée verbally

insulting him and telling him to leave; the Pt said he will not leave bk he pays the rent. Each party called 911 and the police essentially sent the Pt to the ED to separate them for a cooling off period.

Pt is pleasant t/o the interview with all ED staff. Pt presentation appears to chronically dramatic/grandiose which may be consistent with mania, emotional distress, personality d/o, mood d/o, and/or delusional disorder and may be obfuscated by OUD pathology. He was recently observed on AIMH unit x 4 days, and while he was primarily admitted for a depressive episode, suspicion was maintained for a personality contribution. This was reinforced by his rapid improvement without specific intervention. He was maintained on his home lithium, prazosin, and trazodone. Additionally low-dose risperidone was added to target depression, anxiety, and insomnia. Suboxone dosing was not altered during the course of his stay.

Pt does not meet criteria for involuntary admission. Due to lack of acute psychiatric symptoms, available support, independence with ADLs, willingness to seek help, future-orientation, engagement with outpatient psychiatry for medication management and no safety concerns will discharge Pt to home with outpatient f/u already in place s/p discharge from AIMH on 5-1-2023

PLAN:

- Patient is safe for d/c to home with outpatient follow up.
- F/u at his next scheduled outpatient appointment 5-5-2023
- Reviewed Safety Plan and provided with copy.
- Counseling provided re: alternative coping strategies, attending f/u MH appointments and taking medication as prescribed for best outcomes for MH recovery and maintenance.
- Provided Veteran's Crisis Line #. Pt educated re worsening s/sx and advised to call if these occur.
- Pt verbalized understanding and agreement with plan.

This case discussed with ED attending Dr. Cutter and Alisa Meloche LMSW

/es/ NANCY MARIE BIGELOW

Nurse Practitioner, Psychiatry

Signed: 05/04/2023 16:19

Receipt Acknowledged By:

05/05/2023 07:59 /es/ JESICA LEIGH KALMBACH
Staff Psychologist

05/04/2023 16:25 /es/ ALISA J MELOCHE LMSW
Psychiatric Social Worker

05/08/2023 09:38 /es/ MARC A. OLSON, MD
Staff Physician, Substance Abuse Clinic

05/04/2023 ADDENDUM

STATUS: COMPLETED

Writer called back by Emergency Department Physician regarding the care of Mr. Rockenhaus. Previously seen by ER Mental Health Consult team (see notes above), and cleared for d/c. Patient Mr. Rockenhaus reportedly called fiancée (Adrienne Blair 734-585-6277) to tell her he was coming home. Ms. Blair called into the AAVA ED to report that she had safety concerns (text messages saying that he was going to plan on harming himself if SHE had to leave house to go to the hospital

for her own mental health care). Also reported suicide attempt of laying on tracks (of note, this was prior to last admission, and he has not had any suicidal attempts since most recent d/c from inpt admission on 5/1). Ms. Blair was hyperverbal on the phone. Upon re-evaluation, pt continues to deny any SI/HI. He was offered voluntary admission to AIMH for further evaluation and management. Pt declined. Pt does not meet criteria for involuntary admission as he continues to deny symptoms.

Safety plan reviewed with patient. He has close follow-up with SUD-C Psychologist visit tomorrow 5/5 at 9am. Pt reported if he did have symptoms of SI/HI overnight he would discuss at visit tomorrow.

Originally safety planned with patient for him to go to a hotel for the night, so he would be able to make it to 9am appt at AAVA tomorrow morning, and so that he could allow some time for domestic argument to resolve. When writer left to print out safety plan, writer returned to patient talking to Ms. Blair on the phone, saying they had made up and she will pick him up. Writer again reinforced benefits of spending the night apart and recommended hotel for the evening. Writer offered patient 1mg Risperidone to help relax prior to d/c from ED.

/es/ MICHAEL JAMES FRANKLIN
Resident, Psychiatry
Signed: 05/04/2023 18:32

/es/ ANUSHA RANGANATHAN MD
Attending Physician, Psychiatry
Cosigned: 05/05/2023 10:02

SUICIDE RISK EVALUATION - COMPREHENSIVE

Details

Date: May 4, 2023
Location: ANN ARBOR VA MEDICAL CENTER
Written by: ALISA J MELOCHE
Signed by: ALISA J MELOCHE
Date signed: May 4, 2023

Notes

LOCAL TITLE: SUICIDE RISK EVALUATION - COMPREHENSIVE
STANDARD TITLE: SUICIDE PREVENTION RISK ASSESSMENT SCREENING NOT
DATE OF NOTE: MAY 04, 2023@15:32 ENTRY DATE: MAY 04, 2023@15:32:12
AUTHOR: MELOCHE,ALISA J EXP COSIGNER:
URGENCY: STATUS: COMPLETED

*** SUICIDE RISK EVALUATION - COMPREHENSIVE Has ADDENDA ***

Comprehensive Suicide Risk Evaluation

This is an update to an existing suicide risk evaluation.
The validity of the information contained within this evaluation is not in

question.

Suicidal Ideation

The most recent thoughts of engaging in suicide-related behavior were within the past 30 days.

Pt denies SI since he was hospitalized on AIMH 4/26/23.

The Veteran had suicidal intent at the time of the most recent ideation.

Description of intent: acting on SI on 4/26/23 by lying on railroad tracks. Denies SI, plan or intent since then.

The Veteran had a suicide plan at the time of the most recent ideation.

Describe: acting on SI on 4/26/23 by lying on railroad tracks. Denies SI, plan or intent since then.

The most recent suicidal ideation was the most severe ideation within the last 30 days.

The Veteran does not have access to lethal means (firearms)

The Veteran does not have access to other lethal means.

Suicidal Behavior

The Veteran has not made any suicide attempts since the last VA Comprehensive Suicide Risk Evaluation was completed.

The Veteran did not report any prior preparatory behaviors that have not been previously documented.

Warning Signs

The following warning signs are currently present for the Veteran:

None noted

Risk Factors

History of suicidal behavior(s)

Comment: multiple, most recent attempt 4/26/23

Recent psychosocial stressors

Please Describe: relationship problems

History of mental health hospitalization

Please Describe: multiple, most recently April 2023 on AIMH

Psychological conditions or symptoms

Please Describe: bipolar d/o, ptsd, anxiety, depression, opioid use d/o

Protective Factors and Reasons for Living

Access to and engagement with health care

Access to and engagement with mental health care

Reports motivation for mental health treatment

Has meaningful family relationships

Comment: mother

Hope for the future

Clinical Impressions:

The clinical impression of acute risk is Low ACUTE Risk.

As evidenced by: Denies SI, plan or intent.

The clinical impression of chronic risk is High CHRONIC Risk.

As evidenced by: Multiple past suicide attempts and psych admissions, long MH/SUD history, limited coping skills, impulsivity, limited support system.

Suicide Risk Mitigation Plan:

This treatment and care plan was developed in collaboration with the Veteran.

This treatment and care plan was developed in collaboration with additional healthcare providers.

List: Nancy Bigelow, NP

Risk Mitigation Plan:

Strategies for Managing Risk if the Veteran is Currently in the EMERGENCY DEPARTMENT/URGENT CARE CENTER:

Suicide Prevention Coordinator was not alerted for consideration of a Patient Record Flag Category I High Risk for Suicide.

Complete or update Veteran's safety plan

Inform Veteran that they will receive follow-up phone contact until engaged in mental health care.

Obtain additional information from collateral sources

Comment: Plymouth PD

Provide Veteran with phone number for Veteran's Crisis Line: Dial 988 (Press 1), Text to 838255, or Chat

Re-evaluation:

Due to the dynamic nature of some warning signs, risk and protective factors, suicide risk should be routinely re-evaluated. These risk management strategies were chosen to address Veteran's current presentation and feasible treatment options within the system of care. This plan should be re-evaluated over time.

/es/ ALISA J MELOCHE LMSW

Psychiatric Social Worker

Signed: 05/04/2023 15:46

05/05/2023 ADDENDUM

STATUS: COMPLETED

Thank you for completing this CSRE and alerting and collaborating with Suicide Prevention team. The Veteran currently has HRS PRF in chart.

Treatment providers should continue to assess for suicide risk during clinical contacts. Suicide Prevention team will continue to monitor Veteran's care, per HR PRF protocol. SP team appreciates the ongoing collaboration & coordination of care by Veteran's treatment team.

/es/ MARY PAT TUCKER, LISW-S

Suicide Prevention Coordinator

Signed: 05/05/2023 09:44

EMERGENCY DEPT NURSE DISCHARGE

Details

Date: May 4, 2023

Location: ANN ARBOR VA MEDICAL CENTER

Written by: BRANDON THOMAS TEMPLETON

Signed by: BRANDON THOMAS TEMPLETON
Date signed: May 4, 2023

Notes

LOCAL TITLE: EMERGENCY DEPT NURSE DISCHARGE
STANDARD TITLE: EMERGENCY DEPT DISCHARGE NOTE
DATE OF NOTE: MAY 04, 2023@19:00 ENTRY DATE: MAY 04, 2023@19:00:36
AUTHOR: TEMPLETON,BRANDON T EXP COSIGNER:
URGENCY: STATUS: COMPLETED

ER Nurse Discharge Template

Time Discharge:May 4,2023

Room Number: 16

Mode of Departure:

Patient left per self Ambulatory

Patient status:

alert, oriented x 3(person, place and time), sensoria clear, independent,
requires no assistance

VITALS:

Temperature:

98.8 F (37.1 C)

PULSE:

89

RESPIRATION:

16

BLOOD PRESSURE:

134/79

PULSE OXIMETRY: 98 done on RA

Current Pain:

0

Pain Scale Used: Numeric Scale

Patient does not have a problem related to pain.

IV-not applicable

Discharge Patient/Family Education: Physician Discharge instructions reviewed
with the patient/significant other and given a printed copy.

Comment: Patient stated he no longer wants to wait for the Risperdal before
discharge.

/es/ BRANDON THOMAS TEMPLETON
REGISTERED NURSE
Signed: 05/04/2023 19:02

EMERGENCY DEPT DISPOSITION

Details

Date: May 4, 2023
Location: ANN ARBOR VA MEDICAL CENTER
Written by: CHRISTINA MARIE MD CUTTER
Signed by: CHRISTINA MARIE MD CUTTER
Date signed: May 10, 2023

Notes

LOCAL TITLE: EMERGENCY DEPT DISPOSITION
STANDARD TITLE: EMERGENCY DEPT DISCHARGE NOTE
DATE OF NOTE: MAY 04, 2023@15:52 ENTRY DATE: MAY 04, 2023@15:52:57
AUTHOR: CUTTER,CHRISTINA MA EXP COSIGNER:
URGENCY: STATUS: COMPLETED

VA Ann Arbor Healthcare System
2215 Fuller Rd.
Ann Arbor MI 48105
FEB 17, 2026

ROCKENHAUS,CONRAD ALAN
35560 GRAND RIVER AVE # 244
FARMINGTN HLS, MICHIGAN 48335

EMERGENCY DEPT. PATIENT DISCHARGE INSTRUCTIONS

You were evaluated in the emergency department. We conducted laboratory studies and you were evaluated by our mental health colleagues. You have been provided with a safety plan that was conducted several days prior to presentation. It will be very important that you continue to follow the safety plan and follow-up at the appointments as scheduled following your recent admission. Please ensure that your mental health team or primary care provider re-assess your Lithium level. Please return to the emergency department if you experience thoughts of hurting yourself/others or any other symptoms that are of concern to you.

For problems call:

The Ann Arbor VA call center 1-734-845-5500 or
1-800-361-8387 ext. 55500

05/05/2023 09:00 ANN SUD-PHD 5
05/12/2023 09:00 ANN SUD-PHD 5

05/16/2023 09:30 ANN SUD-MD 1
05/16/2023 10:00 ANN VVC-VOD SUD-PSY 3
05/19/2023 09:00 ANN SUD-PHD 5
06/13/2023 11:00 ANN VVC-VOD NEURO MD-SHAT
07/06/2023 09:00 CAN WHS ACUPUNCTURE 1
10/12/2023 10:00 CAN PACT 2 YELLOW-MD 2

Influenza Immunization

Resolution:

No influenza vaccine recorded for this season.

Information:

Reminder Term: VA-INFLUENZA IMM SEASON START DATE

Computed Finding: VA-FileMan Date

07/30/2022 value - 3220730

Influenza season start date: 7/30/2022

Influenza vaccine recorded after this date will resolve this reminder.

Refusals

Reminder Term: VA-INFLUENZA IMM SEASONAL VACCINE FORMULATIONS

04/20/2023@16:54 Reason: PATIENT DECISION, expires 05/20/2023.

Patient has refused all vaccines in the group.

04/21/2023@14:07:55 Reason: PATIENT DECISION, expires 05/21/2023.

Comments: Patient states he got in December and it was not documented.

Patient has refused all vaccines in the group.

Immunization: INFLUENZA, UNSPECIFIED FORMULATION

04/20/2023@16:54 Reason: PATIENT DECISION, expires 05/20/2023.

Patient has refused all vaccines in the group.

04/21/2023@14:07:55 Reason: PATIENT DECISION, expires 05/21/2023.

Comments: Patient states he got in December and it was not documented.

Patient has refused all vaccines in the group.

Precautions

Reminder Term: VA-INFLUENZA IMM SEASONAL VACCINE FORMULATIONS

04/20/2023@18:31:09 Reason: UNSTABLE/EVOLVING NEURO DZ, expires 05/20/2023.

Immunization: INFLUENZA, UNSPECIFIED FORMULATION

04/20/2023@18:31:09 Reason: UNSTABLE/EVOLVING NEURO DZ, expires 05/20/2023.

Medication Reconciliation:

The patient received a printed list of their medications (including local and

remote VA active medications, non-VA medications, medications that expired or

were discontinued within the last 90 days and pending medication orders).

This list was reviewed-reconciled with the patient. Changes in medications were discussed with the patient. A printed list of the new or changed medications was given to the patient. Patient verbalized understanding of the

changes made.

CHRISTINA MARIE CUTTER MD

Attending Physician, Emergency Medicine

ROCKENHAUS,CONRAD ALAN

SUICIDE PREVENTION SAFETY PLAN REVIEW/DECLINE

Details

Date: May 4, 2023
Location: ANN ARBOR VA MEDICAL CENTER
Written by: ALISA J MELOCHE
Signed by: ALISA J MELOCHE
Date signed: May 4, 2023

Notes

LOCAL TITLE: SUICIDE PREVENTION SAFETY PLAN REVIEW/DECLINE
STANDARD TITLE: SUICIDE PREVENTION NOTE
DATE OF NOTE: MAY 04, 2023@15:47 ENTRY DATE: MAY 04, 2023@15:47:15
AUTHOR: MELOCHE,ALISA J EXP COSIGNER:
URGENCY: STATUS: COMPLETED

Veteran and clinician reviewed prior Safety Plan together; no changes indicated at this time.

Date of current Safety Plan: May 1,2023

/es/ ALISA J MELOCHE LMSW
Psychiatric Social Worker
Signed: 05/04/2023 15:47

EMERGENCY DEPARTMENT MENTAL HEALTH CONSULT

Details

Date: May 4, 2023
Location: ANN ARBOR VA MEDICAL CENTER
Written by: NANCY MARIE BIGELOW
Signed by: NANCY MARIE BIGELOW
Date signed: May 4, 2023

Notes

LOCAL TITLE: EMERGENCY DEPARTMENT MENTAL HEALTH CONSULT
STANDARD TITLE: EMERGENCY DEPT NOTE
DATE OF NOTE: MAY 04, 2023@15:41 ENTRY DATE: MAY 04, 2023@15:41:31
AUTHOR: BIGELOW,NANCY MARIE EXP COSIGNER:
URGENCY: STATUS: COMPLETED

*** EMERGENCY DEPARTMENT MENTAL HEALTH CONSULT Has ADDENDA ***

EMERGENCY DEPT MENTAL HEALTH NOTE

PATIENT IDENTIFYING DATA

Name: ROCKENHAUS,CONRAD ALAN

Age: 41

Gender: MALE

Ethnicity: WHITE

Marital status: DIVORCED

Total time: 90 min including F2F with Pt and care coordination.
Pt was interviewed in the ED together with Nancy Bigelow, NP.

DETAILS OF PRESENTING PROBLEM:

Pt is a 41 y/o male with PPHx including Opioid use d/o on MAT, Bipolar d/o, PTSD, anxiety and depression with multiple past suicide attempts and psychiatric hospitalizations (most recently on AIMH April 26, 2023 to May 1, 2023), who presented to the ED via EMS after an alleged argument with his fiancée. Police were called to the residence and Pt reports they gave him a choice of going to jail or coming to the ED.

On interview today he denies any mood disturbance or thoughts of self-harm, suicide, or thoughts of harming others. Pt endorses stable mood since discharge from AIMH on 5-1-2023. He reports more stressors in his relationship since starting couples therapy on 5-2-2023, stating "I feel like she doesn't validate my stressors . . .and she doesn't like when I point this out to the therapist".

The alleged altercation happened today when he says he had a seizure and his fiancée got upset and started "yelling at me" and that "I can't be your caretaker" because she could lose her job. The fiancée then insisted he lie down after his "seizure", and when he awakened, she called him "pathetic" and "a limp-dick man" and wanted him to leave. The Pt reports he then said "I'm not leaving because I pay the rent".

At that point the fiancée called 911, stating that she told the police that he pushed her which Pt denies. Pt then stated that he also called 911 "so they could hear my side of the story". When the police arrived Pt states the police believed that he did not push the fiancée but gave him the ultimatum of "going to jail or going to the hospital to appease her". Reports the police told him he could return back to the residence after being evaluated.

MH team informed the Pt that we will need to verify with the police department that he may return home or what the appropriate steps will be from ED. Pt is agreeable and states he lives in Plymouth, MI and the Plymouth Police

Department was called. The Police department verified they have no concerns for him to be discharged and return to his residence.

Denies any issues with sleep or appetite. Reports compliance with medication regimen and intention to attend post discharge appointments; Pt also reports he is planning on attending all f/u appointments including appointment tomorrow.

PSYCHIATRIC HISTORY:

- Per chart, Dx include Bipolar I (fights & over-spending), opioid use d/o related to chronic back pain (now on suboxone), PTSD, depression, anxiety, possible delusional d/o with paranoid and sometimes grandiose delusions.
- Previous treatment in TX, recently established care in SUD-C here.
- Multiple past psych admissions (at least 12), most recently on AIMH from 4-26-2023 to 5-1-2023.
- Multiple past psych med trials including Ambien, Klonopin, Valium, Ativan, Eszopiclone, Zoloft, Remeron, Effexor, Cymbalta, Doxepin, Nortriptyline, Seroquel, Haldol, Risperdal consta, Geodon, Depakote, Lithium, Buspar, Hydroxyzine, Propranolol, Prazosin, Trazodone, Suboxone.
- Per chart, multiple suicide attempts. Pt discussed today's attempt via lying on railroad tracks, fiance found him by tracking his location on his phone then brought him to the ED. Pt reported the most recent attempt prior to today was in Oct 2022 - ingested ~50 Metformin tablets in the context of difficulty adjusting to civilian life after incarceration, argument w/ fianc?e and opiate use.

SUBSTANCE USE HISTORY:

- OUD on MAT with buprenorphine
- denies cannabis use "I quit" since discharging from AIMH unit on 5-1-2023
- denies etoh, denies illicit drug use
- occasional adderall from a friend who has a prescription

PSYCHOSOCIAL HISTORY AND STRESSORS:

Divorced, 3 children (notes from 4-27-2023 indicated Pt has 2 children). Recently moved from TX to MI and lives with his fiance, they've been together x3 years. Current income from 100% VA SC. Per chart, Pt has a BA in computer science, past employment- eb/security/computer consulting business. Currently on supervised release for 3 years after ~40 months of incarceration for computer hacking. Pt cites his fiance and his mother as main social supports. Independent w/ ADLs. Denied access to firearms.

Military Service - Navy 2003-2015

Percent SC: 100

Rated disabilities:

SCARS 0% SC

BIPOLAR DISORDER 70% SC

TRAUMATIC BRAIN DISEASE 10% SC

LIMITED FLEXION OF THIGH 0% SC

LIMITED MOTION OF ARM 0% SC

HYPERTENSIVE VASCULAR DISEASE 0% SC

SCARS 0% SC

SCARS 0% SC

LIMITED MOTION OF ARM 0% SC

SLEEP APNEA SYNDROMES 50% SC

MIGRAINE HEADACHES 30% SC
LIMITED FLEXION OF THIGH 10% SC
SCARS 0% SC
LUMBOSACRAL OR CERVICAL STRAIN 0% SC
PARALYSIS OF SCIATIC NERVE 10% SC
LUMBOSACRAL OR CERVICAL STRAIN 10% SC
IRRITABLE COLON 0% SC
LIMITED FLEXION OF KNEE 10% SC
TINNITUS 10% SC
SEIZURE DISORDER 100% SC

MEDICAL HISTORY:

Per chart:

- Seizure disorder
- Sleep apnea, not using CPAP ("ex-wife stole it")
- Migraine
- Sciatica
- Chronic back pain
- Tendinitis
- Hypertension
- Neck pain
- Atopic dermatitis
- Hearing loss
- Morbid obesity
- Diabetes
- Allergic rhinitis
- Pulmonary vein injury
- GERD

Active Outpatient Medications (including Supplies):

-
- 1) ACETAMINOPHEN 250/ASA 250/CAFF 65MG TAB TAKE 2 ACTIVE
TABLETS BY MOUTH EVERY DAY AS NEEDED FOR HEADACHE
 - 2) ACETAMINOPHEN 500MG TAB TAKE 1 TABLET BY MOUTH TWICE ACTIVE
A DAY AS NEEDED FOR PAIN
 - 3) BUPRENORPHINE 8MG/NALOXONE 2MG SL TAB DISSOLVE 1 ACTIVE
TABLET UNDER THE TONGUE EVERY MORNING - THIS RX FOR
5-1-23 TO 5-16-23
 - 4) FAMOTIDINE 20MG TAB TAKE ONE TABLET BY MOUTH TWICE A ACTIVE
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 - 5) FOLIC ACID 1MG TAB TAKE ONE TABLET BY MOUTH ONCE ACTIVE
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 - 6) LITHIUM CARBONATE 300MG CAP TAKE FOUR CAPSULES BY ACTIVE
MOUTH AT BEDTIME FOR MENTAL HEALTH
 - 7) PILL SPLITTER-EA USE SPLITTER -- AS DIRECTED TO SPLIT ACTIVE
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 - 9) RISPERIDONE 1MG TAB TAKE ONE-HALF TABLET BY MOUTH ACTIVE
THREE TIMES A DAY AS NEEDED FOR ANXIETY AND
DIFFICULTY SLEEPING
 - 10) TRAZODONE HCL 100MG TAB TAKE ONE TABLET BY MOUTH AT ACTIVE
BEDTIME FOR SLEEP

11) VITAMIN B COMPLEX/VITAMIN C CAP/TAB TAKE 1 TABLET BY ACTIVE MOUTH ONCE DAILY FOR VITAMIN SUPPLEMENT

MENTAL STATUS EXAMINATION:

Alert: sitting up in bed in VA attire good hygiene, good eye contact, pleasant and cooperative in interview

Orientation: A & O x 4

Psychomotor: no psychomotor agitation/retardation. No tremors or tics noted.

Speech: normal rate, rhythm, loud volume x times

Mood: "fine"

Affect: broad, reactive; mood congruent

Thought processes: overall logical, linear, goal-directed

Concentration: appropriate to interview; no deficits noted

Thought content: Denies SI/HI intent or plan. Denies SIB thoughts/urges.

Hallucinations: Denies AVH. No evidence of responding to internal stimuli or thought blocking.

Delusions: No paranoid or delusional statements elicited or expressed.

Insight: limited-to-fair

Judgement: limited-to-fair

SUICIDE RISK:

See CSRE

HOMICIDE/VIOLENCE RISK: No. Plymouth Police Department contacted by MH team to verify they have no safety concerns to discharge Pt to return to the residence; Plymouth PD have no concerns for Pt to be discharged.

DSM V DIAGNOSES:

- mood disorder unspecified d/o
- bipolar d/o by history
- OUD severe on MAT with buprenorphine
- chronic PTSD by history
- Delusional d/o by history
- cannabis use daily (recently quit)

IMPRESSION:

Pt is a 41 y/o male with psychiatric history of bipolar d/o, chronic PTSD, delusional d/o, OUD severe on MAT, past suicide attempts (most recent report of S.A is lying on railroad tracks on 4-26-2023), numerous inpatient psychiatric admissions (most recent AIMH 4-26-2023 to 5-1-2023), has received MH treatment at the inpatient and outpatient levels of care who presents to AAVAED via EMS for purported SI after argument with fiancée.

Pt denies low mood, SI, or HI now or at any time since discharge from AIMH unit on 5-1-2023 and reports that the police gave him the choice of going to jail or going to get evaluated in the ED after 911 was called after argument with fiancée. Pt chose to come to ED to be evaluated and "appease" his fiancée. The Plymouth Police Dept. was contacted by the ED MH team and report each party said they pushed the other and they also confirm there are no safety concerns for the Pt to return to residence.

Pt provided his version of events that led to ED presentation and in summary:

he and fiancée had argument today after he had a "seizure" and she said she could not be his caretaker. The argument escalated to the fiancée verbally insulting him and telling him to leave; the Pt said he will not leave b/c he pays the rent. Each party called 911 and the police essentially sent the Pt to the ED to separate them for a cooling off period.

Pt is pleasant t/o the interview with all ED staff. Pt presentation appears to be chronically dramatic/grandiose which may be consistent with mania, emotional distress, personality d/o, mood d/o, and/or delusional disorder and may be obfuscated by OUD pathology. He was recently observed on AIMH unit x 4 days, and while he was primarily admitted for a depressive episode, suspicion was maintained for a personality contribution. This was reinforced by his rapid improvement without specific intervention. He was maintained on his home lithium, prazosin, and trazodone. Additionally low-dose risperidone was added to target depression, anxiety, and insomnia. Suboxone dosing was not altered during the course of his stay.

Pt does not meet criteria for involuntary admission. Due to lack of acute psychiatric symptoms, available support, independence with ADLs, willingness to seek help, future-orientation, engagement with outpatient psychiatry for medication management and no safety concerns will discharge Pt to home with outpatient f/u already in place s/p discharge from AIMH on 5-1-2023

PLAN:

- Patient is safe for d/c to home with outpatient follow up.
- F/u at his next scheduled outpatient appointment 5-5-2023
- Reviewed Safety Plan and provided with copy.
- Counseling provided re: alternative coping strategies, attending f/u MH appointments and taking medication as prescribed for best outcomes for MH recovery and maintenance.
- Provided Veteran's Crisis Line #. Pt educated re worsening s/sx and advised to call if these occur.
- Pt verbalized understanding and agreement with plan.

This case discussed with ED attending Dr. Cutter and Alisa Meloche LMSW

/es/ NANCY MARIE BIGELOW

Nurse Practitioner, Psychiatry

Signed: 05/04/2023 16:19

Receipt Acknowledged By:

05/05/2023 07:59 /es/ JESICA LEIGH KALMBACH
Staff Psychologist

05/04/2023 16:25 /es/ ALISA J MELOCHE LMSW
Psychiatric Social Worker

05/08/2023 09:38 /es/ MARC A. OLSON, MD
Staff Physician, Substance Abuse Clinic

05/04/2023 ADDENDUM

STATUS: COMPLETED

Writer called back by Emergency Department Physician regarding the care of Mr. Rockenhaus. Previously seen by ER Mental Health Consult team (see notes above), and cleared for d/c. Patient Mr. Rockenhaus reportedly called fiancée (Adrienne Blair 734-585-6277) to tell her he was coming home. Ms. Blair called into the

AAVA ED to report that she had safety concerns (text messages saying that he was going to plan on harming himself if SHE had to leave house to go to the hospital for her own mental health care). Also reported suicide attempt of laying on tracks (of note, this was prior to last admission, and he has not had any suicidal attempts since most recent d/c from inpt admission on 5/1). Ms. Blair was hyperverbal on the phone. Upon re-evaluation, pt continues to deny any SI/HI. He was offered voluntary admission to AIMH for further evaluation and management. Pt declined. Pt does not meet criteria for involuntary admission as he continues to deny symptoms.

Safety plan reviewed with patient. He has close follow-up with SUD-C Psychologist visit tomorrow 5/5 at 9am. Pt reported if he did have symptoms of SI/HI overnight he would discuss at visit tomorrow.

Originally safety planned with patient for him to go to a hotel for the night, so he would be able to make it to 9am appt at AAVA tomorrow morning, and so that he could allow some time for domestic argument to resolve. When writer left to print out safety plan, writer returned to patient talking to Ms. Blair on the phone, saying they had made up and she will pick him up. Writer again reinforced benefits of spending the night apart and recommended hotel for the evening. Writer offered patient 1mg Risperidone to help relax prior to d/c from ED.

/es/ MICHAEL JAMES FRANKLIN
Resident, Psychiatry
Signed: 05/04/2023 18:32

/es/ ANUSHA RANGANATHAN MD
Attending Physician, Psychiatry
Cosigned: 05/05/2023 10:02

SUICIDE RISK EVALUATION - COMPREHENSIVE

Details

Date: May 4, 2023
Location: ANN ARBOR VA MEDICAL CENTER
Written by: ALISA J MELOCHE
Signed by: ALISA J MELOCHE
Date signed: May 4, 2023

Notes

LOCAL TITLE: SUICIDE RISK EVALUATION - COMPREHENSIVE
STANDARD TITLE: SUICIDE PREVENTION RISK ASSESSMENT SCREENING NOT
DATE OF NOTE: MAY 04, 2023@15:32 ENTRY DATE: MAY 04, 2023@15:32:12
AUTHOR: MELOCHE,ALISA J EXP COSIGNER:
URGENCY: STATUS: COMPLETED

*** SUICIDE RISK EVALUATION - COMPREHENSIVE Has ADDENDA ***

Comprehensive Suicide Risk Evaluation

This is an update to an existing suicide risk evaluation.
The validity of the information contained within this evaluation is not in question.

Suicidal Ideation

The most recent thoughts of engaging in suicide-related behavior were within the past 30 days.
Pt denies SI since he was hospitalized on AIMH 4/26/23.
The Veteran had suicidal intent at the time of the most recent ideation.
Description of intent: acting on SI on 4/26/23 by lying on railroad tracks. Denies SI, plan or intent since then.
The Veteran had a suicide plan at the time of the most recent ideation.
Describe: acting on SI on 4/26/23 by lying on railroad tracks. Denies SI, plan or intent since then.
The most recent suicidal ideation was the most severe ideation within the last 30 days.
The Veteran does not have access to lethal means (firearms)
The Veteran does not have access to other lethal means.

Suicidal Behavior

The Veteran has not made any suicide attempts since the last VA Comprehensive Suicide Risk Evaluation was completed.
The Veteran did not report any prior preparatory behaviors that have not been previously documented.

Warning Signs

The following warning signs are currently present for the Veteran:
None noted

Risk Factors

History of suicidal behavior(s)
Comment: multiple, most recent attempt 4/26/23
Recent psychosocial stressors
Please Describe: relationship problems
History of mental health hospitalization
Please Describe: multiple, most recently April 2023 on AIMH
Psychological conditions or symptoms
Please Describe: bipolar d/o, ptsd, anxiety, depression, opioid use d/o

Protective Factors and Reasons for Living

Access to and engagement with health care
Access to and engagement with mental health care
Reports motivation for mental health treatment
Has meaningful family relationships
Comment: mother
Hope for the future

Clinical Impressions:

The clinical impression of acute risk is Low ACUTE Risk.

As evidenced by: Denies SI, plan or intent.

The clinical impression of chronic risk is High CHRONIC Risk.

As evidenced by: Multiple past suicide attempts and psych admissions, long MH/SUD history, limited coping skills, impulsivity, limited support system.

Suicide Risk Mitigation Plan:

This treatment and care plan was developed in collaboration with the Veteran.

This treatment and care plan was developed in collaboration with additional healthcare providers.

List: Nancy Bigelow, NP

Risk Mitigation Plan:

Strategies for Managing Risk if the Veteran is Currently in the EMERGENCY DEPARTMENT/URGENT CARE CENTER:

Suicide Prevention Coordinator was not alerted for consideration of a Patient Record Flag Category I High Risk for Suicide.

Complete or update Veteran's safety plan

Inform Veteran that they will receive follow-up phone contact until engaged in mental health care.

Obtain additional information from collateral sources

Comment: Plymouth PD

Provide Veteran with phone number for Veteran's Crisis Line: Dial 988 (Press 1), Text to 838255, or Chat

Re-evaluation:

Due to the dynamic nature of some warning signs, risk and protective factors, suicide risk should be routinely re-evaluated. These risk management strategies were chosen to address Veteran's current presentation and feasible treatment options within the system of care. This plan should be re-evaluated over time.

/es/ ALISA J MELOCHE LMSW

Psychiatric Social Worker

Signed: 05/04/2023 15:46

05/05/2023 ADDENDUM

STATUS: COMPLETED

Thank you for completing this CSRE and alerting and collaborating with Suicide Prevention team. The Veteran currently has HRS PRF in chart. Treatment providers should continue to assess for suicide risk during clinical contacts. Suicide Prevention team will continue to monitor Veteran's care, per HR PRF protocol. SP team appreciates the ongoing collaboration & coordination of care by Veteran's treatment team.

/es/ MARY PAT TUCKER, LISW-S

Suicide Prevention Coordinator

Signed: 05/05/2023 09:44

EMERGENCY DEPT FLOWSHEET

Details

Date: May 4, 2023
Location: ANN ARBOR VA MEDICAL CENTER
Written by: BRANDON THOMAS TEMPLETON
Signed by: BRANDON THOMAS TEMPLETON
Date signed: May 4, 2023

Notes

LOCAL TITLE: EMERGENCY DEPT FLOWSHEET
STANDARD TITLE: EMERGENCY DEPT NOTE
DATE OF NOTE: MAY 04, 2023@14:54 ENTRY DATE: MAY 04, 2023@14:54:56
AUTHOR: TEMPLETON,BRANDON T EXP COSIGNER:
URGENCY: STATUS: COMPLETED

ER flowsheet

Falls Risk:
Patient is considered a high risk for falls ? No
Emergency Department high risk falls prevention program protocol initiated? No

VITALS:

PULSE OXIMETRY: done on RA

Current Pain:

0

Pain Scale Used: Numeric Scale

Review of Systems at time of Admission

Lab/Test:

Labs Drawn

Time Drawn:

Treatments:

Time	Treatment Description
------	-----------------------

Patient aox4 w/ clear speech. Patient screened positive on columbia screening but denies SI/HI at this time and is calm/cooperative. Patient states he was in an altercation with wife last night that spilled over into today when PD were called. Patient is calm and cooperative at this time and denies drug or ETOH use.

IMMUNIZATIONS:

/es/ BRANDON THOMAS TEMPLETON
REGISTERED NURSE
Signed: 05/04/2023 14:56

EMERGENCY DEPARTMENT TRIAGE

Details

Date: May 4, 2023
Location: ANN ARBOR VA MEDICAL CENTER
Written by: BRANDON THOMAS TEMPLETON
Signed by: BRANDON THOMAS TEMPLETON
Date signed: May 4, 2023

Notes

LOCAL TITLE: EMERGENCY DEPARTMENT TRIAGE
STANDARD TITLE: NURSING EMERGENCY DEPT NOTE
DATE OF NOTE: MAY 04, 2023@14:34 ENTRY DATE: MAY 04, 2023@14:34:50
AUTHOR: TEMPLETON,BRANDON T EXP COSIGNER:
URGENCY: STATUS: COMPLETED

*** EMERGENCY DEPARTMENT TRIAGE Has ADDENDA ***

EMERGENCY DEPT NURSING TRIAGE

Check-in Date/Time: 05/04/23 13:55

Triage Date/Time: MAY 04, 2023 14:34

The patient was asked if in the last 14 days they have had new onset of any COVID-19 symptoms. They report the following:

No symptoms

Within the past 14 days, the patient reports no exposure to someone with a febrile/respiratory illness or someone with a known or suspected case of COVID-19 (within 6 feet for > 15 minutes).

Result:

Screen is negative.

Patient age:41 Sex: MALE

On arrival patient was: AMBULANCE

Patient phone number: PATIENT PHONE

Allergies: SULFA DRUGS, DARUNAVIR

Subjective/Chief Complaint:

Pt to ED w/ cc of SI from home after altercation with wife and local PD were called.

Objective:

Current Medications:

Active Outpatient Medications (including Supplies):

Active Outpatient Medications

Status

-
- 1) ACETAMINOPHEN 250/ASA 250/CAFF 65MG TAB TAKE 2 ACTIVE
TABLETS BY MOUTH EVERY DAY AS NEEDED FOR HEADACHE
 - 2) ACETAMINOPHEN 500MG TAB TAKE 1 TABLET BY MOUTH TWICE ACTIVE
A DAY AS NEEDED FOR PAIN
 - 3) BUPRENORPHINE 8MG/NALOXONE 2MG SL TAB DISSOLVE 1 ACTIVE
TABLET UNDER THE TONGUE EVERY MORNING - THIS RX FOR
5-1-23 TO 5-16-23
 - 4) FAMOTIDINE 20MG TAB TAKE ONE TABLET BY MOUTH TWICE A ACTIVE
DAY FOR HEARTBURN
 - 5) FOLIC ACID 1MG TAB TAKE ONE TABLET BY MOUTH ONCE ACTIVE
DAILY
 - 6) LITHIUM CARBONATE 300MG CAP TAKE FOUR CAPSULES BY ACTIVE
MOUTH AT BEDTIME FOR MENTAL HEALTH
 - 7) PILL SPLITTER-EA USE SPLITTER -- AS DIRECTED TO SPLIT ACTIVE
TABLETS
 - 8) PRAZOSIN HCL 1MG CAP TAKE THREE CAPSULES BY MOUTH ACTIVE
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 - 9) RISPERIDONE 1MG TAB TAKE ONE-HALF TABLET BY MOUTH ACTIVE
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BEDTIME FOR SLEEP
 - 11) VITAMIN B COMPLEX/VITAMIN C CAP/TAB TAKE 1 TABLET BY ACTIVE
MOUTH ONCE DAILY FOR VITAMIN SUPPLEMENT

Current Problems: ACTIVE PROBLEMS

Vital Signs *

Temperature

98.8 F (37.1 C)

Pulse

100

Respirations

16

Blood Pressure

138/82

Pain scale recorded:

0

Pulse Oximetry 98 Room Air

SEPSIS SCREENING

Is the patient's history suggestive of a new infection?

No

Sepsis screening not indicated, will continue to monitor.

The patient is not a fall risk.

Suicide Screen:

Columbia Suicide Severity Rating Scale (C-SSRS) screener

Specific method of suicide recently considered; this POSITIVE answer requires same-day completion of a Suicide Risk Evaluation-Comprehensive

1. Over the past month, have you wished you were dead or wished you could go to sleep and not wake up?

Yes

2. Over the past month, have you had any actual thoughts of killing yourself?

Yes

3. Over the past month, have you been thinking about how you might do this?

Yes

4. Over the past month, have you had these thoughts and had some intention of acting on them?

No

5. Over the past month, have you started to work out or worked out the details of how to kill yourself?

No

6. If yes, at any time in the past month did you intend to carry out this plan?

Response not required due to responses to other questions.

7. In your lifetime, have you ever done anything, started to do anything, or prepared to do anything to end your life (for example, collected pills, obtained a gun, gave away valuables, went to the roof but didn't jump)?

Yes

8. If YES, was this within the past 3 months?

No

Patient had a C-SSRS that was positive. Warm handoff for CSRE.

Hand off to: Russ, ED tech

HOMICIDE SCREENING

Have you had any recent thoughts of hurting someone else?

No

SUSPECTED ASSAULT, ABUSE OR NEGLECT SCREEN

The patient has not exhibited any clinical signs and/or stated that